



Stomach II — Polyps & Gastric Cancer

Pathology GIT Block · MEDucation by Mattin Majid · Practice MCQs

HOW TO USE THIS SET

20 questions, mixed difficulty, 9 case-based. Attempt every question before checking the answer key on the last page.

4 Easy

10 Medium

6 Hard

1.

EASY

What is the commonest histological type of gastric cancer?

- A. Lymphoma
- B. Adenocarcinoma
- C. Gastrointestinal stromal tumour
- D. Squamous cell carcinoma
- E. Carcinoid tumour

2.

EASY

The stomach is the commonest site for which tumour type outside lymph nodes?

- A. Adenocarcinoma
- B. Sarcoma
- C. Extranodal non-Hodgkin lymphoma
- D. Melanoma
- E. Carcinoid tumour

3.

EASY

Metastatic gastric carcinoma to the ovary is called:

- A. Blumer shelf
- B. Virchow node
- C. Krukenberg tumour
- D. Sister Mary Joseph nodule
- E. Irish node

4.

EASY

Which gastric polyp type is the commonest overall?

- A. Adenomatous
- B. Fundic gland
- C. Hyperplastic / inflammatory
- D. Hamartomatous
- E. Peutz-Jeghers

5.

MEDIUM

CASE

A gastrectomy specimen shows diffuse thickening and rigidity of the stomach wall with flattened rugae — a "leather bottle" appearance. Histology shows individual malignant cells with abundant intracytoplasmic mucin pushing the nucleus to the periphery.

Which statement about this tumour is correct?

- A. It typically arises from a recognised precursor lesion
- B. It has no known precursor lesion and no established risk factors
- C. It grows as an exophytic mass projecting into the lumen
- D. It forms glands resembling colonic adenocarcinoma
- E. It is the intestinal-type tumour described by Lauren

6.

MEDIUM

CASE

A different gastrectomy specimen shows an exophytic, ulcerated mass. Histology shows malignant cells forming recognisable glandular structures, arising adjacent to an area of intestinal metaplasia.

Which Lauren type does this represent, and what follows from that classification?

- A. Diffuse type — no precursor lesion, so screening cannot help
- B. Intestinal type — has a recognised precursor and risk factors, making it the type screening and H. pylori eradication can target
- C. Diffuse type — associated with signet ring morphology, which this case shows
- D. Intestinal type — always presents with linitis plastica
- E. Neither type — glandular morphology excludes Lauren classification entirely

7.

MEDIUM

Which gastric polyp is defined by the presence of dysplasia, and what follows from that definition?

- A. Hyperplastic polyp — low malignant potential
- B. Fundic gland polyp — associated with FAP
- C. Adenomatous polyp — definite risk of harbouring adenocarcinoma
- D. Inflammatory polyp — arises in chronic gastritis
- E. Hamartomatous polyp — associated with Peutz-Jeghers syndrome

8.

MEDIUM

CASE

A patient on long-term proton pump inhibitor therapy is found to have multiple small gastric polyps on endoscopy.

Which polyp type is most associated with chronic PPI use?

- A. Hyperplastic polyp
- B. Fundic gland polyp
- C. Adenomatous polyp
- D. Inflammatory polyp
- E. Peutz-Jeghers polyp

9.

MEDIUM

Gastric adenocarcinoma incidence fell by more than 85% during the 20th century. Which factor is most credited with this decline?

- A. Increased use of proton pump inhibitors
- B. Reduced dietary N-nitroso compounds and benzo[a]pyrene as salting/smoking of food declined, plus earlier detection via screening
- C. Wider use of subtotal gastrectomy for benign disease
- D. Rising consumption of pickled vegetables
- E. Declining rates of H. pylori testing

10.

MEDIUM

CASE

Endoscopy shows a gastric ulcer with heaped-up, irregular margins and a dirty, necrotic base, in contrast to a benign peptic ulcer seen in another patient with regular margins, radiating folds, and a clean base.

Which single feature most strongly suggests malignancy in the first ulcer?

- A. Its solitary nature
- B. Its location on the lesser curvature
- C. The heaped-up, irregular borders
- D. Its size under 2 cm
- E. The presence of radiating folds

11.

MEDIUM

CASE

A patient with gastric adenocarcinoma is found to have a firm periumbilical nodule on examination.

Which eponym applies to this finding?

- A. Virchow node
- B. Sister Mary Joseph nodule
- C. Irish node
- D. Blumer shelf
- E. Krukenberg tumour

12.

MEDIUM

On rectal examination, a patient with gastric carcinoma is found to have a firm shelf-like mass palpable anteriorly. Which eponym describes this?

- A. Virchow node
- B. Sister Mary Joseph nodule
- C. Irish node
- D. Blumer shelf
- E. Krukenberg tumour

13.

MEDIUM

Which dietary factor is associated with DECREASED risk of gastric cancer?

- A. Smoked or cured fish
- B. Pickled vegetables
- C. Large amounts of starch
- D. Fresh vegetables rich in vitamin C
- E. Nitrosamine-containing preserved meats

14.

MEDIUM

A left supraclavicular lymph node enlarged by metastatic gastric carcinoma is known by which eponym?

- A. Krukenberg tumour
- B. Virchow node
- C. Blumer shelf
- D. Sister Mary Joseph nodule
- E. Irish node

15.

HARD

CASE

A patient has an enlarged left axillary lymph node found to contain metastatic gastric adenocarcinoma, in addition to a periumbilical nodule found on the same examination.

Which two eponyms correctly apply to these two findings respectively?

- A. Irish node (axilla) and Sister Mary Joseph nodule (umbilicus)
- B. Virchow node (axilla) and Blumer shelf (umbilicus)
- C. Irish node (axilla) and Krukenberg tumour (umbilicus)
- D. Sister Mary Joseph nodule (axilla) and Irish node (umbilicus)
- E. Blumer shelf (axilla) and Virchow node (umbilicus)

16.

HARD

Which statement correctly contrasts the Lauren intestinal and diffuse types of gastric adenocarcinoma?

- A. Both types are equally likely to present as linitis plastica
- B. The diffuse type has a recognised precursor lesion; the intestinal type does not
- C. The intestinal type shows signet ring cells; the diffuse type forms glands
- D. The diffuse type shows a desmoplastic stromal reaction producing rugal flattening, which the intestinal type does not characteristically show
- E. Risk factors are equally well established for both types

17.

HARD CASE

A polyp is removed from the gastric antrum. Pathology reports it as showing dysplastic glandular epithelium.

Based on this description alone, which management implication follows?

- A. No further action needed — this describes a hyperplastic polyp, which carries low risk
- B. This describes an adenomatous polyp, which carries a definite risk of harbouring adenocarcinoma and needs assessment for that
- C. This describes a fundic gland polyp and warrants FAP screening only, with no cancer risk from the polyp itself
- D. This describes an inflammatory polyp related to chronic gastritis, requiring only gastritis treatment
- E. Dysplasia in a gastric polyp is never clinically significant unless the polyp exceeds 2 cm

18.

HARD CASE

A tumour is described as arising without a recognised precursor lesion and without established risk factors, showing signet ring morphology and a marked desmoplastic reaction.

Which statement best explains why this tumour type cannot currently be prevented by screening or H. pylori eradication, unlike its intestinal-type counterpart?

- A. It progresses too quickly for any screening interval to catch it
- B. It lacks the identifiable precursor stage and risk factor exposure that screening and eradication strategies are designed to interrupt
- C. It is not associated with H. pylori at all, so eradication is irrelevant to either type
- D. Screening is equally effective for both types; this is a common misconception
- E. It only affects patients too young to qualify for screening programmes

19.

HARD CASE

A gastric ulcer is being assessed endoscopically. The clinician needs to decide, based on gross morphology alone, whether it is more likely benign or malignant before biopsy results return.

Which combination of features would most strongly point toward a BENIGN process?

- A. Heaped-up, irregular borders with a necrotic base
- B. Regular margins, radiating mucosal folds, and a clean base
- C. Diffuse wall thickening with flattened rugae
- D. A periumbilical nodule found on abdominal exam
- E. Bilateral ovarian masses found on imaging

20.

HARD CASE

A patient has three findings on work-up: a firm left supraclavicular node, a shelf-like rectal mass on digital exam, and bilateral solid ovarian masses. All three are confirmed to be metastatic gastric adenocarcinoma.

Which three eponyms correctly correspond to these findings, in the order given?

- A. Virchow node, Blumer shelf, Krukenberg tumour
- B. Irish node, Sister Mary Joseph nodule, Blumer shelf
- C. Virchow node, Krukenberg tumour, Blumer shelf
- D. Sister Mary Joseph nodule, Blumer shelf, Irish node
- E. Blumer shelf, Virchow node, Krukenberg tumour

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	B	Adenocarcinoma accounts for over 90% of gastric cancers.
2	C	The stomach is the commonest extranodal site for non-Hodgkin lymphoma.
3	C	Krukenberg tumour is metastatic gastric carcinoma to the ovary, classically bilateral.
4	C	Hyperplastic/inflammatory polyps are the commonest gastric polyp type, arising in chronic gastritis.
5	B	Linitis plastica with signet ring cells is the diffuse type, which has no recognised precursor lesion or established risk factors — unlike the intestinal type.
6	B	Glandular morphology arising near intestinal metaplasia is the intestinal type, which has a precursor and risk factors — the type screening and H. pylori eradication can target.
7	C	Adenomatous polyps are defined by dysplasia and carry a definite risk of harbouring adenocarcinoma, unlike hyperplastic, fundic gland, or inflammatory polyps.
8	B	Fundic gland polyps are associated with FAP and with chronic PPI use.
9	B	Reduced dietary N-nitroso compounds/benzo[a]pyrene from less salting and smoking of food, plus earlier detection through screening, are credited with the decline.
10	C	Heaped-up, irregular borders with a necrotic base are the classic gross features distinguishing a malignant ulcer from a benign one.
11	B	A periumbilical metastatic nodule is a Sister Mary Joseph nodule.
12	D	A shelf-like mass palpable in the pouch of Douglas/anteriorly on rectal exam is Blumer shelf.
13	D	Fresh vegetables rich in vitamin C and antioxidants, along with whole milk, are associated with decreased gastric cancer risk.
14	B	Left supraclavicular nodal metastasis is Virchow node (Troisier sign).
15	A	Left axillary metastasis is Irish node; periumbilical metastasis is Sister Mary Joseph nodule.
16	D	The diffuse type's desmoplastic reaction stiffens the wall and flattens rugae (linitis plastica); the intestinal type does not characteristically show this.
17	B	Dysplastic glandular epithelium defines an adenomatous polyp, which carries a definite risk of harbouring adenocarcinoma and needs further assessment/excision.
18	B	The diffuse type lacks the identifiable precursor lesion and risk factor exposures that make prevention strategies like screening and H. pylori eradication effective for the intestinal type.
19	B	Regular margins, radiating folds, and a clean base are the classic benign ulcer features, in contrast to the heaped-up, necrotic appearance of malignancy.
20	A	Left supraclavicular node = Virchow node; rectal shelf = Blumer shelf; bilateral ovarian masses = Krukenberg tumour.

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